

Statistical Analysis Plan

Protocol STUDY01 - Drug A versus Placebo

Version 1.0 (synthetic, for SpecGen testing)

1. Introduction

This Statistical Analysis Plan (SAP) describes the planned analyses for Protocol STUDY01, a randomised, double-blind, placebo-controlled study of Drug A in adult subjects. It is a synthetic document written to exercise SpecGen; it does not describe a real study.

2. Study Objectives

The primary objective is to assess the safety and tolerability of Drug A 50 mg and Drug A 100 mg compared with placebo. Vital signs are assessed as a secondary safety objective.

3. Study Design

Subjects are randomised 1:1:1 to Placebo, Drug A 50 mg or Drug A 100 mg and treated for approximately 8 weeks (Period 1). Subjects are followed for safety after the last dose of study treatment.

4. Analysis Populations

4.1 Randomised Population

All subjects randomised, analysed by planned treatment (TRT01P).

4.2 Intent-to-Treat (ITT) Population

All randomised subjects. Flag ITTFL = 'Y' in ADSL.

4.3 Safety Population

All randomised subjects who received at least one dose of study treatment, analysed by treatment actually received (TRT01A). Flag SAFFL = 'Y' in ADSL. All demographic and safety tables use the Safety Population.

4.4 Subjects excluded

A subject randomised but never dosed is in the ITT Population and not in the Safety Population.

4.5 Big N

The N shown in each column header is the number of subjects in the analysis population of that table for that treatment group. For Safety Population tables, N is the count of SAFFL = 'Y' subjects in ADSL by TRT01A. The same N must be used in every Safety Population table.

5. Baseline and Visit Windows

5.1 Baseline definition

Baseline is defined per treatment period: the last non-missing assessment on or before the first dose of that period. ABLFL = 'Y' flags the baseline record within each period, so a subject in more than one period has one baseline record per period.

5.2 Visit windows

Assessments are assigned to analysis visits using the windows below (study day, where Day 1 is the first dose).

Visit	Target day	Window (study days)
Baseline	1	Up to Day 1
Week 4	29	22 to 35
Week 8	57	50 to 63

6. Safety Analyses

6.1 Adverse events

Adverse events are coded with MedDRA and summarised by System Organ Class (SOC) and Preferred Term (PT), by treatment received.

6.2 Treatment-emergent adverse events (TEAE)

A TEAE is an adverse event with onset on or after the date of first dose of study treatment and on or before the date of last dose of study treatment + 7 days. TRTEMFL = 'Y' flags these events in ADAE. Events starting before the first dose, or more than 7 days after the last dose, are not treatment-emergent.

6.3 Maximum severity

For a subject with more than one TEAE, the maximum severity is the highest of MILD, MODERATE, SEVERE among that subject's TEAEs.

6.4 Relationship to study drug

An event is related if the investigator assessment (AEREL) is POSSIBLE or PROBABLE.

7. Statistical Methods

7.1 Continuous variables

Summarised with n, mean, standard deviation (SD), median, minimum and maximum.

7.2 Categorical variables

Summarised with n and percentage. The denominator for a percentage is the Big N of the column (section 4.5). A count of zero is shown as 0 with no percentage.

7.3 Subject counts

A subject with more than one event is counted once per SOC, once per PT, and once overall.

8. Reporting Conventions

8.1 *Decimal places*

Mean and median are shown to one more decimal place than the collected data; SD to two more. Minimum and maximum use the precision of the collected data. Percentages are shown to one decimal place.

8.2 *Population and titles*

Each table carries its population under the title and its source dataset and program in the footnotes.

9. Table Inventory

The following tables are required for the clinical study report. Each must be delivered.

Table	Title	Population
14.1.1	Summary of Demographic and Baseline Characteristics	Safety
14.3.1	Overview of Treatment-Emergent Adverse Events	Safety
14.3.2	Treatment-Emergent Adverse Events by System Organ Class and Preferred Term	Safety